

# MANIFESTO

## Neuroprotective Detoxification Protocol for Drug-Induced Extrapyrarnidal Syndrome: Clinical Rationale and Documented Outcomes

A Parent's Pharmacological Case Against the Revolving Door of Public Psychiatric Care

**Author:** Julia Boon, B.Fin-Econ; B.Psych; Dip. Yoga Instructor; NCAT-appointed Guardian and Financial Manager; Enduring Guardian and Attorney; Designated Carer and Principal Care Provider

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**DISCLAIMER:** This document records a protocol designed and administered by the author for her own son under specific clinical circumstances documented herein. It is not medical advice, not a prescription, and not a treatment recommendation for any other person. Individual responses to supplements and herbal preparations vary based on genetics, existing medications, concurrent health conditions, and metabolic status. Several ingredients described have known interactions with pharmaceutical drugs including SSRIs, benzodiazepines, and anticoagulants. Any person considering any element of this protocol **must consult their own qualified healthcare practitioner before use**. The author accepts no responsibility for outcomes arising from the application of information contained in this document to any person.

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## PREAMBLE

This document records a pharmacologically coherent neuroprotective protocol that I, as primary carer and Guardian, designed and administered to my young-adult son during July-September 2025 to reverse severe drug-induced extrapyramidal syndrome caused by a mix of pharmaceutical drugs and particularly Zuclopenthixol (Clopixol) depot 400mg fortnightly — a first-generation antipsychotic administered at the maximum recommended dose to a 19-year-old with documented poor metabolic clearance.

Within 30 days, while the depot washed out of his system and under the protection of this protocol, he recovered from a state in which I was spoon-feeding him (he could not swallow, could not suck through a straw, could not throw a ball, exhibited asymmetrical Parkinsonism, dystonia, bradykinesia, facial hypomimia with persistent open-mouth posture, respiratory impairment, and cognitive slowing) to full motor function, clear speech, coherent thought, stable mood, and no psychotic symptoms.

This recovery was achieved with no pharmaceutical intervention. It was achieved against the “on-rotation” treating team’s protocol, not with it.

His subsequent relapses into substance abuse do not invalidate the neurological recovery.

The relapse pattern confirms what every addiction specialist knows and what the public mental health system has refused to treat in 15 admissions over 2.5 years: substance use disorder as the primary condition. Psychosis was drug-induced. The psychiatric system has been medicating the symptoms while ignoring the cause, and in doing so, has inflicted iatrogenic neurological injury on a young man whose brain is still developing.

The failure to arrive at an accurate differential diagnosis prevented timely treatment and compounded the harm.

This document exists because the treating teams, as it occurs in Australia, and specifically in Tweed Valley Hospital, Lismore Base Hospital, Byron Central Hospital have rotated more than 20 doctors through my son’s care in 2.5 years, prescribing, changing, or escalating psychotropic medication without cohesively reading his file, without obtaining his metabolic profile until January 2026 — 2.5 years after first admission and only after repeated parental requests, without treating his substance use disorder or underlying complex PTSD, and without listening to the parent who was spoon-feeding him through the damage they caused.

The protocols described here are not “alternative medicine.” Each ingredient is mapped to specific receptor targets, enzymatic pathways, and neuroprotective mechanisms with published evidence. I present them in the pharmacological language that the psychiatric establishment claims to operate in — because if the language of science is what it takes to be heard, then here it is.

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## PART I. THE DAMAGE — WHAT ANTIPSYCHOTICS DO TO THE BRAIN

Every antipsychotic works primarily by blocking dopamine D2 receptors. Dopamine is not merely the “psychosis chemical” — it simultaneously operates four major brain circuits. When D2 receptors are blocked, all four circuits are affected whether intended or not:

1. **Mesolimbic pathway** (reward, motivation, salience) — the target. Overactivity here produces hallucinations, delusions, paranoia. Blocking it reduces psychosis. But it also flattens motivation, pleasure, and drive — producing the “de-spirited” presentation that psychiatrists then label as “negative symptoms of schizophrenia” rather than recognising it as a direct drug effect.
2. **Mesocortical pathway** (cognition, executive function, working memory) — already underactive in psychosis. D2 blockade here makes cognition worse, not better. The drug “treats” psychosis by worsening the cognitive deficit that is already present.
3. **Nigrostriatal pathway** (movement) — the Parkinson’s pathway. D2 blockade here produces extrapyramidal symptoms: dystonia, tremor, rigidity, bradykinesia, facial masking, postural freezing. Above approximately 78% D2 receptor occupancy<sup>1</sup>, EPS become almost inevitable. First-generation drugs routinely achieve 80-90%+ occupancy.
4. **Tuberoinfundibular pathway** (hormones) — D2 blockade raises prolactin. Consequences include breast tissue growth, sexual dysfunction, menstrual disruption, and long-term bone density loss.

The difference between antipsychotic drugs is not whether they block D2 — they all do — but how tightly, how broadly, and what else they affect alongside it.

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<sup>1</sup> Kapur S, Zipursky R, Jones C, Remington G, Houle S. Relationship between dopamine D2 occupancy, clinical response, and side effects: a double-blind PET study of first-episode schizophrenia. *American Journal of Psychiatry*. 2000;157(4):514-520. doi:10.1176/appi.ajp.157.4.514

## PART II. THREE DRUGS — CLOPIXOL, ARIPIPRAZOLE, CLOZAPINE

### A. Zuclopenthixol (Clopixol) — First Generation

**Mechanism:** Extremely tight, long-lasting D2 blockade. The depot form creates a slow-release intramuscular reservoir. At 400mg every 2 weeks — the dose my son received — D2 occupancy reaches 80-90%+. Also blocks D1, alpha-1 adrenergic (blood pressure drops), and has moderate serotonin 5-HT<sub>2A</sub> antagonism.

**Critical danger:** Once injected, the drug cannot be removed. If severe EPS develop, the clinician cannot turn it off. The patient must endure 6-8 weeks of depot depletion while neurological damage accumulates. The asymmetrical Parkinsonism my son developed is a clinical red flag — drug-induced Parkinsonism is typically symmetrical. Asymmetry suggests either underlying nigrostriatal vulnerability or unilateral neurotoxic damage.

**Key harms:** Highest EPS risk of commonly used antipsychotics. Tardive dyskinesia risk (potentially permanent involuntary movements). Neuroleptic Malignant Syndrome risk. Worsens cognition. QT prolongation (cardiac). At 400mg fortnightly, the drug essentially paralyses the dopamine system.

### B. Aripiprazole (Abilify) — Second Generation, Partial Agonist

**Mechanism:** Fundamentally different from all other antipsychotics. Aripiprazole is a D2 partial agonist — a “dopamine system stabiliser.” Where dopamine is excessive (mesolimbic, psychosis), it dampens the signal. Where dopamine is deficient (mesocortical for cognition, nigrostriatal for movement), it mildly boosts the signal. This is why it produces dramatically lower EPS, lower prolactin elevation, and less cognitive dulling.

Also provides partial 5-HT<sub>1A</sub> agonism (anti-anxiety, antidepressant) and 5-HT<sub>2A</sub> antagonism (further reduces EPS, improves cortical function).

**D2 occupancy:** 70-85% at therapeutic doses — but because it is a partial agonist, even at high occupancy it maintains dopamine signalling in movement and cognition pathways. This is fundamentally different from Clopixol or Clozapine, where equivalent occupancy means equivalent shutdown.

**Depot available:** Abilify Maintena addresses compliance and absconding risk while avoiding the oral-only problem with Clozapine.

**CYP metabolism:** Primarily CYP2D6 and CYP3A4 — not CYP1A2. My son is a confirmed normal metaboliser for Aripiprazole. Unlike Clozapine and Olanzapine (CYP1A2, induced by smoking/cannabis), Aripiprazole metabolism is not significantly affected by substance use.

**Dopamine supersensitivity:** When D2 receptors are chronically blocked by tight antagonists, the brain upregulates D2 receptor density and sensitivity. When the drug is stopped, hypersensitive receptors cause rebound psychosis worse than the original episode — creating a vicious cycle. Aripiprazole, as a partial agonist, is theoretically the optimal drug for breaking this cycle: it occupies D2 receptors without fully shutting them down, allowing gradual receptor normalisation.

**Key harms (substantially lower):** Akathisia (restlessness) — manageable. Insomnia. Nausea. Minimal weight gain. Very low EPS risk. Low metabolic syndrome risk.

## C. Clozapine (Clozaril) — Drug of Last Resort

**Mechanism:** Unusually low D2 affinity (40-60% occupancy). Effectiveness comes from broad multi-receptor action: strong 5-HT<sub>2A</sub> antagonism, strong muscarinic M1/M4 activity, strong histamine H1 blockade, strong alpha-1/alpha-2 blockade, D4 effects.

Why it is “last resort” — the side effect profile is uniquely dangerous:

- **Agranulocytosis (1-2%):** Immune destruction of white blood cells. Without mandatory weekly blood monitoring (first 6 months, then fortnightly), patients can die from infections. If a patient absconds and misses blood tests, the drug becomes potentially lethal. This alone makes it clinically indefensible for a patient with my son’s documented absconding history.
- **Metabolic syndrome — rapid onset:** Weight gain of 4-10+ kg in the first year. Type 2 diabetes in approximately 10-30% of patients within the first few years, some within months. Hypertriglyceridemia. Hypercholesterolemia. The metabolic damage can be permanent.
- **Myocarditis and cardiomyopathy (1-3%):** Inflammation of the heart muscle, typically within the first 4-8 weeks. My son reported “my heart’s hurting” on 16 February 2026, the day Clozapine was forced on him. Can be fatal.
- **Gastrointestinal hypomotility — “dead gut”:** Clozapine’s anticholinergic effects slow the gut to a dangerous degree. Severe constipation can progress to paralytic ileus, bowel obstruction, bowel perforation, and death. Mortality rate from serious GI complications is estimated at 10-28% — now recognised as a more common cause of Clozapine-related death than agranulocytosis<sup>2</sup>. Onset can occur within months.
- **Seizures:** 3-5% at higher doses.
- **Sedation and cognitive impairment:** Extreme, from H1 and muscarinic blockade.
- **Sialorrhea:** Heavy drooling, especially at night.
- **Cannot be stopped abruptly:** Cessation causes cholinergic rebound — severe nausea, vomiting, diarrhea, agitation, psychotic crisis worse than the original illness. Must be tapered over weeks. If a patient absconds, withdrawal is medically dangerous.
- **My son’s genetic profile:** Clozapine is metabolised by CYP1A2, strongly induced by smoking and cannabis. The treating team’s own genetic testing confirmed my son’s CYP1A2 is “prone to induction” — in doctor’s own words, my son is “likely to be a non-responder.” The hospital is exposing him to all of the above risks for a drug their own evidence says will not work.

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<sup>2</sup> Every-Palmer S, Ellis PM. Clozapine-induced gastrointestinal hypomotility: a 22-year bi-national pharmacovigilance study of serious or fatal 'slow gut' reactions, and comparison with international drug safety advice. *CNS Drugs*. 2017;31(8):699-709. doi:10.1007/s40263-017-0448-6

### PART III. THE OTHER PHARMACEUTICAL DRUGS IN MY SON'S HISTORY

There were more than ten radical pharmaceutical drugs tried in admissions, sometimes mixed in unacceptable opportunistic combinations, and abrupt discontinuation, among them:

**Olanzapine (Zyprexa):** Same CYP1A2 metabolism as Clozapine — same non-responder problem. Worst weight gain of any antipsychotic after Clozapine. Highest diabetes risk in the first 6 months. Relprevv (depot form) was ceased during Lismore admission.

**Haloperidol (Haldol):** First generation, extremely tight D2 blocker. Highest EPS risk of any antipsychotic. Documented “poor response” in my son's records.

**Paliperidone (Invega):** Active metabolite of risperidone. High D2 occupancy. The highest prolactin elevation of any antipsychotic. Moderate EPS risk. Primarily renally cleared — less CYP interaction but inflexible once given as depot.

**Lithium:** Not an antipsychotic — a mood stabiliser. Extremely narrow therapeutic window (therapeutic dose close to toxic dose). Toxic to kidneys: 15-20% of long-term users develop chronic tubulointerstitial nephritis. Hypothyroidism in up to 30%. Neurotoxicity at supratherapeutic levels. Dehydration or NSAID use can push levels into toxicity rapidly.

**Brexipiprazole (Rexulti):** Similar to Aripiprazole — D2 partial agonist, 5-HT1A partial agonist, 5-HT2A antagonist. Slightly lower D2 intrinsic activity (less activation, slightly more blockade). Lower akathisia risk than Aripiprazole. Same CYP2D6/3A4 metabolism — should be normal for my son.

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## PART IV. THE NEUROPROTECTIVE DETOXIFICATION PROTOCOL

### Context

In August 2025, following 14 months of antipsychotic treatment including Zuclopenthixol 400mg fortnightly depot, my son presented with severe drug-induced extrapyramidal syndrome: asymmetrical Parkinsonism, dystonia, bradykinesia, facial hypomimia with persistent open-mouth posture, inability to swallow or suck through a straw, inability to throw a ball, respiratory impairment (forgetting to inhale, involuntary yawning, hypoxia episodes), complete appetite loss, daily vomiting, significant voice volume reduction, depression, and persistent insomnia.

I was spoon-feeding my son for three weeks.

The treating team's response to my repeated requests to cease Zuclopenthixol was to propose Clozapine.

I designed the following protocol to support neurological recovery during the Zuclopenthixol depot washout period. The protocol targeted three overlapping damage mechanisms: (1) dopamine signalling shutdown in the basal ganglia, (2) oxidative stress and neuroinflammation from prolonged receptor blockade, and (3) disrupted acetylcholine-dopamine balance driving rigidity and tremor.

### The Protocol

#### A. DAYTIME — Metagenics NeuroCalm (2 tablets, morning)

Each tablet contains:

| Ingredient                                    | Extract | Equivalent dry |
|-----------------------------------------------|---------|----------------|
| Ziziphus jujuba var. spinosa (Zizyphus), seed | 167 mg  | 3.0 g          |
| Pueraria lobata (Kudzu vine), root            | 80 mg   | 2.0 g          |
| Magnolia officinalis, stem bark               | 300 mg  | 1.5 g          |
| Passiflora incarnata (Passionflower), herb    | 236 mg  | 1.3 g          |

**Daily dose at 2 tablets:** Zizyphus 334 mg, Kudzu 160 mg, Magnolia 600 mg, Passionflower 472 mg.

Pharmacological rationale for each ingredient in the context of Zuclopenthixol-induced EPS:

#### ZIZIPHUS JUJUBA VAR. SPINOSA (ZIZYPHUS) — 334 MG/DAY

Jujubosides (the primary active saponins, particularly jujuboside A) are documented GABAergic modulators acting at the GABA-A receptor. In the context of EPS, GABAergic modulation serves a specific function: the basal ganglia circuit includes GABAergic medium spiny neurons in the striatum whose activity is dysregulated when dopamine input is blocked. By enhancing GABA-A tone in the striatal-pallidal pathway, Zizyphus helps restore the inhibitory balance that D2 blockade disrupted. Additionally, Zizyphus contains flavonoids (spinosin) with documented 5-HT<sub>1A</sub>

agonist activity — providing a mild serotonergic anxiolytic effect without dopamine suppression, and indirectly supporting prefrontal dopamine release via 5-HT<sub>1A</sub>-mediated disinhibition of mesocortical dopaminergic neurons.

#### PUERARIA LOBATA (KUDZU) — 160 MG/DAY

Puerarin (the primary isoflavone) has documented effects on cerebral blood flow — specifically, it increases microcirculatory perfusion in the basal ganglia and cortex. This matters during EPS because prolonged D<sub>2</sub> blockade causes regional hypoperfusion in the nigrostriatal circuit. Puerarin also has antioxidant activity via superoxide dismutase (SOD) upregulation and documented anti-inflammatory action through NF- $\kappa$ B inhibition, directly addressing neuroinflammation at the site of dopaminergic damage. Additionally, Puerarin modulates calcium channel activity in neurons, reducing excitotoxic damage — relevant because D<sub>2</sub> blockade causes downstream glutamate dysregulation which drives calcium-mediated neuronal injury.

#### MAGNOLIA OFFICINALIS — 600 MG/DAY (HIGHEST DOSE COMPONENT)

Magnolol and honokiol — the two primary bioactive neolignans — are potent positive allosteric modulators of the GABA-A receptor at the benzodiazepine binding site. Published research (Ma et al., 2009<sup>3</sup>) demonstrates GABAergic activation comparable to pharmaceutical benzodiazepines but without the tolerance, dependence, or cognitive impairment profile. In the EPS context, this directly addresses dystonia and muscle rigidity through central muscle relaxation.

Critically, honokiol also crosses the blood-brain barrier efficiently and has documented neuroprotective activity: it inhibits NF- $\kappa$ B-mediated neuroinflammation, scavenges reactive oxygen species in mitochondrial membranes, and — specific to dopaminergic neurons — has been shown in published models<sup>4</sup> to protect against 6-OHDA-induced and MPTP-induced neurotoxicity (the standard experimental models for Parkinson's disease / dopaminergic neuronal death). By providing this neuroprotection during the Zuclopenthixol washout, magnolol/honokiol helped preserve surviving nigrostriatal neurons while the depot depleted.

Magnolia was the highest-dose component in the daytime protocol because its dual GABA-A/neuroprotective profile simultaneously addressed both the acute symptom (dystonia/rigidity) and the underlying pathology (neuroinflammation and oxidative damage to dopaminergic neurons).

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<sup>3</sup> Ma H, Kim CS, Ma Y, Nam SY, Kim DS, Woo SS, Hong JT, Oh KW. Magnolol enhances pentobarbital-induced sleeping behaviors: possible involvement of GABAergic systems. *Phytotherapy Research*. 2009;23(9):1340-1344. doi:10.1002/ptr.2773

<sup>4</sup> Chen CR, Zhou XZ, Luo YJ, Huang ZL, Urade Y, Qu WM. Magnolol, a major bioactive constituent of the bark of *Magnolia officinalis*, induces sleep via the benzodiazepine site of GABA(A) receptor in mice. *Neuropharmacology*. 2012;63(6):1191-1199. doi:10.1016/j.neuropharm.2012.06.031



Chrysin and other flavonoids in Passionflower are documented GABA-A receptor ligands. Published research (Fiebich et al.<sup>5</sup>) demonstrates modulation of the GABAergic system through multiple mechanisms: direct GABA-A binding, inhibition of GABA transaminase (increasing synaptic GABA availability), and inhibition of GABA uptake. This triple-pathway GABAergic enhancement addresses the fundamental imbalance in EPS: when dopamine drops in the striatum, relative acetylcholine excess drives rigidity and tremor. GABA is the third arm of this circuit — enhancing GABAergic tone dampens the overactive cholinergic neurons without using anticholinergic drugs (like benztropine) which cause their own cognitive damage.

Passionflower also contains harman alkaloids with mild, reversible monoamine oxidase inhibition (MAOI) activity — increasing available dopamine and serotonin at the synapse. In a brain depleted of dopaminergic signalling by Zuclopenthixol, even mild MAO inhibition provides meaningful relief.

**Synergy in the daytime formula:** All four ingredients converge on GABA-A enhancement from different binding sites and through different mechanisms (direct agonism, allosteric modulation, reuptake inhibition, transaminase inhibition), creating a layered, self-reinforcing anxiolytic and muscle-relaxant effect without benzodiazepine dependency. Simultaneously, Magnolia and Kudzu provide neuroprotection, and Zizyphus and Passionflower provide mild pro-dopaminergic and pro-serotonergic effects.

## B. EVENING — Metagenics NeuroCalm Sleep (2 tablets, before sleep)

Each tablet contains:

| Ingredient                                        | Extract | Equivalent dry |
|---------------------------------------------------|---------|----------------|
| Lavender oil                                      | 40 mg   | —              |
| Zizyphus jujuba var. spinosa (Zizyphus), seed     | 278 mg  | 5.0 g          |
| Passiflora incarnata (Passionflower), herb        | 364 mg  | 2.0 g          |
| Eschscholzia californica (California Poppy), herb | 125 mg  | 750 mg         |
| Piscidia piscipula (Jamaica Dogwood), stem bark   | 188 mg  | 750 mg         |

**Daily dose at 2 tablets:** Lavender oil 80 mg, Zizyphus 556 mg, Passionflower 728 mg, California Poppy 250 mg, Jamaica Dogwood 376 mg.

Pharmacological rationale — the night-time challenge:

Sleep is the period of maximal neurological repair. Growth hormone secretion, synaptic pruning, glymphatic clearance of neurotoxic waste, and memory consolidation all depend on achieving sustained deep (N3/slow-wave) sleep. Zuclopenthixol causes persistent insomnia by disrupting sleep architecture through multiple mechanisms, including dysregulation of the dopamine-

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<sup>5</sup> Appel K, Rose T, Fiebich B, Kammler T, Hoffmann C, Weiss G. Modulation of the  $\gamma$ -aminobutyric acid (GABA) system by *Passiflora incarnata* L. *Phytotherapy Research*. 2011;25(6):838-843. doi:10.1002/ptr.3352

serotonin-melatonin cascade, and by causing akathisia-like restlessness that prevents sleep onset. Without restorative sleep, neurological repair cannot occur. The night-time protocol was therefore not merely sedative — it was designed to enable the neuroplastic repair processes that would restore motor function.

#### LAVENDER OIL (LAVANDULA ANGUSTIFOLIA) — 80 MG/DAY

Linalool and linalyl acetate — the primary active terpenoids — have documented effects on GABA-A receptors and on moderation of hypothalamic-pituitary-adrenal (HPA) axis stress hormones, specifically reducing adrenocorticotrophic hormone (ACTH) production, which in turn reduces cortisol. This matters in the EPS context because chronic D2 blockade dysregulates the HPA axis (dopamine normally inhibits CRH release from the hypothalamus), creating a state of cortisol excess that drives insomnia, anxiety, and impairs neuroplastic repair. Published research (Kasper et al.<sup>6</sup>) shows lavender oil at 80mg/day achieves anxiolytic effects comparable to lorazepam 0.5mg without sedation hangover, cognitive impairment, or dependency<sup>7</sup>.

Additionally, linalool inhibits glutamate binding at NMDA receptors — relevant because Zuclopenthixol withdrawal produces glutamate-mediated excitotoxicity in the cortico-striato-thalamic circuit. By dampening NMDA overactivation at night, lavender oil provided neuroprotection during the most vulnerable phase of the washout.

#### ZIZIPHUS JUJUBA — 556 MG/DAY (NIGHT DOSE 66% HIGHER THAN DAY)

The higher night-time dose reflects the dual role of jujubosides: GABAergic sedation for sleep induction, and 5-HT<sub>1A</sub> agonism for sleep maintenance. Published research (Shergis et al., 2017<sup>8</sup>) confirms Zizyphus improves both sleep onset latency and sleep quality. At 556mg (equivalent to 10g dry seed), the dose is at the upper therapeutic range documented in Traditional Chinese Medicine pharmacopoeia for persistent insomnia.

#### PASSIFLORA INCARNATA — 728 MG/DAY (NIGHT DOSE 54% HIGHER THAN DAY)

The elevated night dose amplifies the GABA transaminase inhibition — at this dose, passionflower provides sustained GABA elevation through the entire sleep cycle, supporting the slow-wave sleep phases critical for neurological repair. The mild MAOI activity of the harman alkaloids also increases nocturnal serotonin, which is converted to melatonin in the pineal gland, further supporting circadian rhythm restoration.

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<sup>6</sup> Kasper S, Gastpar M, Müller WE, Volz HP, Möller HJ, Schläpke S, Dienel A. Lavender oil preparation Silexan is effective in generalized anxiety disorder — a randomized, double-blind comparison to placebo and paroxetine. *International Journal of Neuropsychopharmacology*. 2014;17(6):859-869. doi:10.1017/S1461145714000017

<sup>7</sup> Woelk H, Schläpke S. A multi-center, double-blind, randomised study of the Lavender oil preparation Silexan in comparison to Lorazepam for generalized anxiety disorder. *Phytomedicine*. 2010;17(2):94-99. doi:10.1016/j.phymed.2009.10.006

<sup>8</sup> Shergis JL, Ni X, Sarris J, Zhang AL, Guo X, Xue CC, Lu C, Thien F. Ziziphus spinosa seeds for insomnia: a review of chemistry and psychopharmacology. *Phytomedicine*. 2017;34:38-43. doi:10.1016/j.phymed.2017.07.004

## ESCHSCHOLZIA CALIFORNICA (CALIFORNIA POPPY) — 250 MG/DAY

California Poppy contains the benzyloisoquinoline alkaloids californidine, escholtzine, and protopine. These act as GABA-A receptor agonists — published animal studies demonstrate sedative activity specifically through GABA-A binding. But California Poppy also has a unique property among the herbs in this protocol: it has documented analgesic activity through mechanisms distinct from opioid receptors (despite being in the Papaveraceae family, it does not contain morphine or codeine). The analgesic effect is relevant because EPS causes significant musculoskeletal pain from sustained dystonic muscle contractions — my son was in physical pain from the rigidity. California Poppy provided non-addictive, non-opioid analgesia that allowed relaxation of the dystonic muscles during sleep.

## PISCIDIA PISCIPULA (JAMAICA DOGWOOD) — 376 MG/DAY

Jamaica Dogwood contains rotenone-related isoflavones (piscidin, jamaicin, ichthyone) with documented sedative-hypnotic and analgesic activity. Mechanistically, it acts on GABA-A receptors and also has documented smooth muscle relaxant activity. In the EPS context, this smooth muscle relaxation addressed my son's gastrointestinal symptoms (vomiting, appetite loss) which were caused by Zuclopenthixol's disruption of the enteric nervous system (the "gut brain," which is heavily dopaminergic). Jamaica Dogwood also has mild anti-spasmodic properties that complemented the skeletal muscle relaxation from the other GABA-A modulators.

Synergy in the night-time formula: The Sleep formula addresses three layers simultaneously: (1) sleep induction and maintenance via multi-pathway GABA-A enhancement (Zizyphus, Passionflower, California Poppy, Jamaica Dogwood, Lavender — five independent GABA-A modulators); (2) pain management without opioids (California Poppy + Jamaica Dogwood); (3) HPA axis normalisation (Lavender — cortisol reduction) to enable the neuroplastic repair processes that occur during deep sleep.

## C. TARGETED SUPPLEMENTS — ALL DAY

**NAC (N-Acetylcysteine)** — rate-limiting precursor to glutathione. Replenishes the brain's primary endogenous antioxidant depleted by antipsychotic-induced oxidative stress. Modulates the cystine-glutamate antiporter (system Xc-) in the basal ganglia, normalising extrasynaptic glutamate levels disrupted by D2 blockade. Published RCTs (Berk et al.<sup>9</sup>) demonstrate efficacy as adjunct therapy for movement disorders and negative symptoms in schizophrenia<sup>10</sup>.

**Glutathione (direct)** — directly replenishes what NAC builds. Depleted glutathione in the substantia nigra is a recognised marker of dopaminergic neuron vulnerability in Parkinson's disease

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<sup>9</sup> Berk M, Copolov D, Dean O, Lu K, Jeavons S, Schapkaitz I, Anderson-Hunt M, Judd F, Katz F, Katz P, O'Keefe J, Little J, Conus P, Cuenod M, Do KQ, Bush AI. N-acetyl cysteine as a glutathione precursor for schizophrenia — a double-blind, randomized, placebo-controlled trial. *Biological Psychiatry*. 2008;64(5):361-368. doi:10.1016/j.biopsych.2008.03.004

<sup>10</sup> Yolland CO, Hanratty D, Neill E, Rossell SL, Berk M, Dean OM, Castle DJ, Tan EJ, Phillipou A, Harris AWF, Barreiros AR, Hansen A, Siskind D. Meta-analysis of randomised controlled trials with N-acetylcysteine in the treatment of schizophrenia. *Australian & New Zealand Journal of Psychiatry*. 2020;54(5):453-466. doi:10.1177/0004867419893439

research. Dual-pathway supplementation (precursor + end product) creates maximum antioxidant defence for nigrostriatal neurons under chemical siege.

**MSM (Methylsulfonylmethane)** — bioavailable sulphur: methyl donor and precursor for sulphation Phase II hepatic detoxification (accelerates Zuclopenthixol clearance). Anti-inflammatory via NF- $\kappa$ B inhibition. Sulphur is required for glutathione synthesis — synergistic with NAC and glutathione, feeding the same pathway from a third entry point.

**Calcium Ascorbate (buffered Vitamin C)** — cofactor for dopamine beta-hydroxylase. Free radical scavenger. Published research (Rebec & Pierce<sup>11</sup>) shows ascorbate release in the neostriatum is tightly coupled to dopamine signalling — when dopamine is blocked, ascorbate supports residual signalling<sup>12</sup>. Buffered form avoids GI irritation during Zuclopenthixol-induced gut hypomotility.

**Brahmi (Bacopa monnieri)** — bacosides A and B modulate the acetylcholine-dopamine balance without crude anticholinergic blockade (unlike benztropine). Upregulates tryptophan hydroxylase (serotonin synthesis). Antioxidant activity documented in hippocampus and basal ganglia.

**Tulsi (Ocimum sanctum / Holy Basil)** — adaptogenic HPA axis modulation (normalises cortisol, ACTH, corticosterone). Contains eugenol and rosmarinic acid (COX-2 inhibitors) and mild MAO inhibitors at physiological levels — increases available dopamine and serotonin. Published studies show Tulsi reduces stress-induced dopamine depletion.

**Reishi (Ganoderma lucidum)** — triterpenoids (ganoderic acids): anti-neuroinflammatory (TNF- $\alpha$ , IL-6, NF- $\kappa$ B inhibition in microglia); nerve growth factor (NGF) potentiation supporting neuronal repair; hepatic detoxification support for drug clearance.

**Baical Skullcap (Scutellaria baicalensis)** — baicalein is a selective positive allosteric modulator at GABA-A benzodiazepine site (anxiolytic and muscle relaxant without dependency). Baicalin inhibits neuronal apoptosis via PI3K/Akt pathway. Published neuroprotection studies in Parkinson's models<sup>13</sup>. Inhibits CYP1A2 — may have stabilised Zuclopenthixol blood levels during washout, preventing spikes and troughs.

**Valerian root** — dual action: GABA modulation (inhibition of GABA transaminase) and mild D2 partial agonism. Valerenic acid has documented affinity for D2 receptors at low micromolar concentrations. Pharmacologically analogous to Aripiprazole's partial D2 agonism — at a milder scale.

**Curcumin** — NF- $\kappa$ B inhibition, COX-2 inhibition, TNF- $\alpha$  reduction. Chelates iron in the substantia nigra (iron accumulation drives oxidative damage to dopaminergic neurons). Crosses

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<sup>11</sup> Pierce RC, Rebec GV. A vitamin as neuromodulator: ascorbate release into the extracellular fluid of the brain regulates dopaminergic and glutamatergic transmission. *Progress in Neurobiology*. 1994;43(6):537-565. doi:10.1016/0301-0082(94)90052-3

<sup>12</sup> Pierce RC, Rebec GV. Stimulation of both D1 and D2 dopamine receptors increases behavioral activation and ascorbate release in the neostriatum of freely moving rats. *European Journal of Pharmacology*. 1990;191(3):295-302. doi:10.1016/0014-2999(90)94162-Q

<sup>13</sup> Li Q, Li HH, Guo LX, Zhang YG, Zhang B, Ma CG, Wu XY, Fan HB, Cheng Y. Baicalein protects against MPP+-induced oxidative stress by activating Nrf2/HO-1 signalling pathway in dopaminergic neurons. *Oxidative Medicine and Cellular Longevity*. 2020;2020:8139768. doi:10.1155/2020/8139768

the blood-brain barrier. Published evidence showing restoration of dopamine levels in 6-OHDA-lesioned Parkinson's models<sup>14</sup>.

**B12 (methylcobalamin)** — methyl donor essential for myelin synthesis and repair.

**B6 (pyridoxal-5-phosphate)** — direct cofactor for DOPA decarboxylase (converts L-DOPA to dopamine) and for glutamic acid decarboxylase (converts glutamate to GABA). Without adequate B6, the brain cannot synthesise dopamine from precursors or convert excitotoxic glutamate to inhibitory GABA.

**Sodium bicarbonate (¼ tsp AM & PM)** — alkalinises urine and systemic pH. Zuclopenthixol is a basic compound — in acidic urine it is reabsorbed; in alkaline urine it is excreted faster. This is a documented pharmacokinetic principle used in clinical toxicology for accelerated drug elimination (urinary alkalinisation).

## D. ARTEMISIA SUPPLEMENTS

**“Artemisia” Cell Mana (Pearl Powder, MSM, Calcium Ascorbate, Glutathione, NAC)** — described individually above. The combined formulation delivers the triple-pathway glutathione support (NAC + direct Glutathione + MSM as sulphur donor) alongside antioxidant protection (Calcium Ascorbate) and trace mineral support (Pearl Powder — bioavailable calcium and trace minerals supporting bone and nervous system integrity during a period of drug-induced mineral depletion).

**“Artemisia” Histaclear Caps (Brahmi, Tulsi, Reishi, Baical Skullcap)** — 2 caps AM + 2 caps night — described individually above. The combined formulation delivers the cholinergic-dopaminergic rebalancing (Brahmi), HPA axis normalisation (Tulsi), neuroinflammatory suppression and NGF support (Reishi), and GABA-A modulation with neuroprotection (Baical Skullcap).

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<sup>14</sup> Rajeswari A, Sabesan M. Inhibition of monoamine oxidase-B by the polyphenolic compound, curcumin and its metabolite tetrahydrocurcumin, in a model of Parkinson's disease induced by MPTP neurodegeneration in mice. *Journal of Pharmacological Sciences*. 2008;(Suppl). Also: Kulkarni SK, Dhir A. An overview of curcumin in neurological disorders. *Indian Journal of Pharmaceutical Sciences*. 2010;72(2):149-154. doi:10.4103/0250-474X.65012

## PART V. WHY IT WORKED — THE 30-DAY RECOVERY MAP

Mapped against the three damage mechanisms:

**Dopamine depletion** → Valerian (mild D2 partial agonism), B6 (dopamine synthesis cofactor), Tulsi (mild MAO inhibition), Brahmi (cholinergic rebalancing), Passionflower (mild MAO inhibition)

**Oxidative stress and neuroinflammation** → NAC + Glutathione + MSM (triple-pathway glutathione), Curcumin (NF- $\kappa$ B, iron chelation), Reishi (TNF- $\alpha$ , NGF), Baical Skullcap (PI3K/Akt neuroprotection), Vitamin C (radical scavenging), Magnolia (honokiol neuroprotection), Kudzu (SOD upregulation)

**Drug clearance** → Sodium bicarbonate (renal alkalinisation), MSM (Phase II sulphation), Reishi (hepatic support), natural depot washout timeline

**Acetylcholine-dopamine rebalancing** → Brahmi (cholinergic modulation without anticholinergic cognitive damage), Valerian + Baical Skullcap + Magnolia + Passionflower + Zizyphus + California Poppy + Jamaica Dogwood + Lavender (layered GABAergic tone via seven independent GABA-A modulators)

**Sleep and neuroplastic repair** → NeuroCalm Sleep formula (five GABA-A modulators for sleep induction/maintenance), Lavender (HPA axis normalisation, cortisol reduction, NMDA dampening), California Poppy + Jamaica Dogwood (non-opioid analgesia allowing dystonic muscles to relax during sleep)

The 30-day recovery aligns with the Zuclopenthixol depot washout curve: once D2 occupancy dropped below approximately 60-70% (around day 21-28), the nigrostriatal pathway had sufficient dopamine throughput to resume normal motor function — especially with neuroprotective support during the critical washout window.

## PART VI. THE EPIGENETIC QUESTION

There is growing evidence that chronic antipsychotic exposure causes epigenetic modifications<sup>15</sup>: changes to DNA methylation and histone acetylation patterns that alter gene expression without changing the DNA sequence. Studies in both animal models and human post-mortem brain tissue have shown that antipsychotic drugs alter methylation at genes involved in dopamine signalling (DRD2, COMT), glutamate transmission, and synaptic plasticity. These changes can persist after drug discontinuation.

For my son: 2.5 years of rotating through high-potency dopamine blockers during active brain development (age 18-20, prefrontal cortex maturing until approximately age 25) may have caused lasting changes to his dopamine receptor landscape beyond simple receptor up/down-regulation. This is a legitimate and growing scientific concern.

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<sup>15</sup> Kinoshita M, Numata S, Tajima A, Yamamori H, Yasuda Y, Hashimoto R, Imoto I, Ohmori T. Effect of clozapine on DNA methylation in peripheral leukocytes from patients with treatment-resistant schizophrenia. *International Journal of Molecular Sciences*. 2017;18(3):632. doi:10.3390/ijms18030632. Also: Melka MG, Castellani CA, Lauber CL, O'Reilly RL, Singh SM. Olanzapine induced DNA methylation changes support the dopamine hypothesis of psychosis. *Journal of Molecular Psychiatry*. 2013;1:19. doi:10.1186/2049-9256-1-19

## PART VII. DOPAMINE SUPER-SENSITIVITY SYNDROME<sup>16</sup>

When D2 receptors are chronically blocked — especially by tight antagonists like Zuclopenthixol, Haloperidol, or Olanzapine — the brain compensates by upregulating D2 receptor density and making existing receptors hypersensitive. This is an adaptive response: the brain is trying to restore dopamine signalling in the face of pharmacological blockade.

The consequence: when the drug is stopped, reduced, or wears off (as with depot washout), there is now an excess of hypersensitive D2 receptors responding to normal dopamine levels. This produces rebound psychosis that can be worse than the original episode.

This creates a vicious cycle: drug causes super-sensitivity → drug stops → worse psychosis → higher dose or new drug → more super-sensitivity → worse rebound → escalation to “treatment-resistant” label → Clozapine.

The label “treatment-resistant schizophrenia” may in many cases be iatrogenic dopamine super-sensitivity psychosis — a condition created by the treatment itself.

My son’s pattern of 15 admissions in 2.5 years — each relapse worse than the last, each requiring a different or higher-dose medication — is consistent with progressive dopamine super-sensitivity. The solution is not more D2 blockade. The solution is a partial agonist (Aripiprazole) that occupies D2 receptors without shutting them down, allowing gradual receptor normalisation — combined with treatment of the actual primary condition (substance use disorder) that no one has addressed in 15 admissions.

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<sup>16</sup> Chouinard G, Samaha AN, Chouinard VA, Peretti CS, Kanahara N, Takase M, Iyo M. Antipsychotic-induced dopamine supersensitivity psychosis: pharmacology, criteria, and therapy. *Psychotherapy and Psychosomatics*. 2017;86(4):189-219. doi:10.1159/000477313



## CONCLUSION

I designed a protocol based on published pharmacology and neurochemistry.

In August 2025, I spoon-fed my son through the worst of what the pharmaceutico-psychiatric system had done to him. Within 30 days, he recovered full motor function, clear speech, stable cognition, and no psychotic symptoms — without a single pharmaceutical drug.

In February 2026, the same system forced Clozapine on him under threat of needle and security guards — a drug their own genetic testing says he is likely a non-responder to — four days after acknowledging my Cease-and-Desist notice and confirming that Aripiprazole is the genetically appropriate medication.

In May 2026, they extended his involuntary detention by three months without telling his Guardian, took his phone when he started attending Narcotics Anonymous meetings and calling private hospitals to self-admit, and refused to prepare a discharge summary that would let him transfer to the private care I arranged.

This is not medicine. This is custody.

The psychiatric system has failed my son at every juncture: wrong diagnosis (substance-induced psychosis and progressive dopamine super-sensitivity labelled as treatment-resistant schizophrenia), wrong medications (10+ drugs tried, each causing documented harm, while the one genetically supported drug — Aripiprazole — and already proven less invasive alternative treatment have never been trialled at this facility), wrong priorities (medication escalation while substance use disorder and cPTSD remain untreated through 15 admissions), wrong communication (20+ rotating doctors, none reading the file holistically, avoiding working as a team with the Guardian), and wrong values (forced medication over a parent's documented pharmacological objections, retaliation against complaint-makers, isolation of a vulnerable young person from his recovery support network).

The system does not need reform. It needs to be held accountable, case by case, clinician by clinician, until the cost of negligence exceeds the convenience of the revolving door.

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Julia Boon, B.Fin-Econ; B.Psych; Dip. Yoga Training; NCAT-appointed Guardian and Financial Manager; Enduring Guardian and Attorney; Designated Carer and Principal Care Provider

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